OFFICIAL CLINICAL ASSESSMENT DOCUMENT

GONSTEAD CHIROPRACTIC CLINICAL PLATFORM

Spinal Health Assessment Report

Comprehensive Digital Spinal Screening · Patient-Facing Clinical Record

ASSESSMENT DATE
19 April 2026

GENERATED
09:42 AM PKT

REPORT ID
RPT-2026-04-GC-00847

CONFIDENTIAL PATIENT RECORD

SUPERVISING CLINICIAN **Dr. Bashir Khan** *DC · Gonstead Certified Practitioner*

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EXECUTIVE SUMMARY & CLINICAL INTERPRETATION

CLINICIAN'S SUMMARY

"This patient presents with a moderate-to-high risk spinal profile requiring timely clinical intervention. Findings indicate multi-level involvement across the lumbar and cervical regions, with concurrent lifestyle-related contributors."

Digital spinal screening conducted via the Gonstead Clinical Assessment Protocol. Results reflect self-reported symptom data, postural indicators, and lifestyle risk factors. All findings require in-person physical and radiographic evaluation before any clinical diagnosis or treatment protocol is initiated. This is a screening tool only — not a diagnostic report.

RISK PROFILE

COMPOSITE SPINAL RISK SCORE



⚠ MODERATE-HIGH RISK

PRIORITY CASE

Consultation recommended within **48–72 hours**

PATIENT PROFILE

FULL NAME	Mr. Hassan Tariq
DATE OF BIRTH	14 March 1988
AGE	38 years
BIOLOGICAL SEX	Male
OCCUPATION	Software Engineer
ASSESSMENT MODE	Digital Screening

PRIMARY SPINAL FINDINGS

- **Lumbar (L4–L5)** SIGNIFICANT
  Recurring lower back pain with radiating discomfort to left leg. Probable disc involvement. Sitting aggravates.
- **Cervical (C5–C7)** MODERATE
  Intermittent neck stiffness, restricted ROM. Tension headaches 3–4× weekly. Likely postural origin.
- **Thoracic (T6–T8)** MILD
  Mild mid-back tightness. No acute pain. Consistent with extended desk-based working.

CONTRIBUTING RISK FACTORS

Sedentary Occupational Posture	88%
Chronic Pain Duration (>6 months)	76%
Insufficient Physical Activity	65%
Sleep Disturbance from Pain	55%
Prior Spinal Injury History	40%

RED-FLAG REVIEW

NEUROLOGICAL WARNING DETECTED

Numbness and tingling in the left lower extremity. This pattern may indicate nerve root compression and warrants urgent clinical evaluation to rule out serious pathology.

NO SYSTEMIC RED FLAGS PRESENT

No indicators of bowel/bladder dysfunction, unexplained weight loss, fever, or trauma. Systemic pathology not suggested by current data.

RECOMMENDED NEXT STEPS — CLINICAL ACTION PLAN

01 **Schedule In-Person Consultation**
Contact Gonstead Chiropractic within 48 hrs for a comprehensive physical and neurological examination.

02 **Radiographic Spinal Imaging**
Full-spine X-ray under the Gonstead Protocol to assess structural alignment and disc integrity.

03 **Formal Neurological Assessment**
Neurological screen required before commencing any manual therapy given left-leg radiculopathy.

04 **Postural & Ergonomic Evaluation**
Workplace ergonomics review advised. Seated posture corrections reduce ongoing spinal loading.

05 **Lifestyle Modification Guidance**
Personalized activity modification, movement protocols, and restorative sleep positioning guidance.

06 **Ongoing Progress Monitoring**
Follow-up digital assessment at 4 weeks post-consultation to track symptom progression.

"At Gonstead Chiropractic, we are committed to evidence-informed, patient-centred spinal care — delivered with precision, compassion, and clinical integrity."

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CONSULTATIONS
By Appointment Only

SCREENING DISCLAIMER — This document is a digital health screening result only and does not constitute a medical diagnosis, clinical opinion, or treatment recommendation. All findings must be reviewed by a licensed practitioner before any clinical decisions are made. If experiencing severe or emergency symptoms — including loss of bladder/bowel control, acute chest pain, or sudden neurological deficit — seek immediate medical attention. Gonstead Chiropractic accepts no liability for decisions made without formal clinical review.